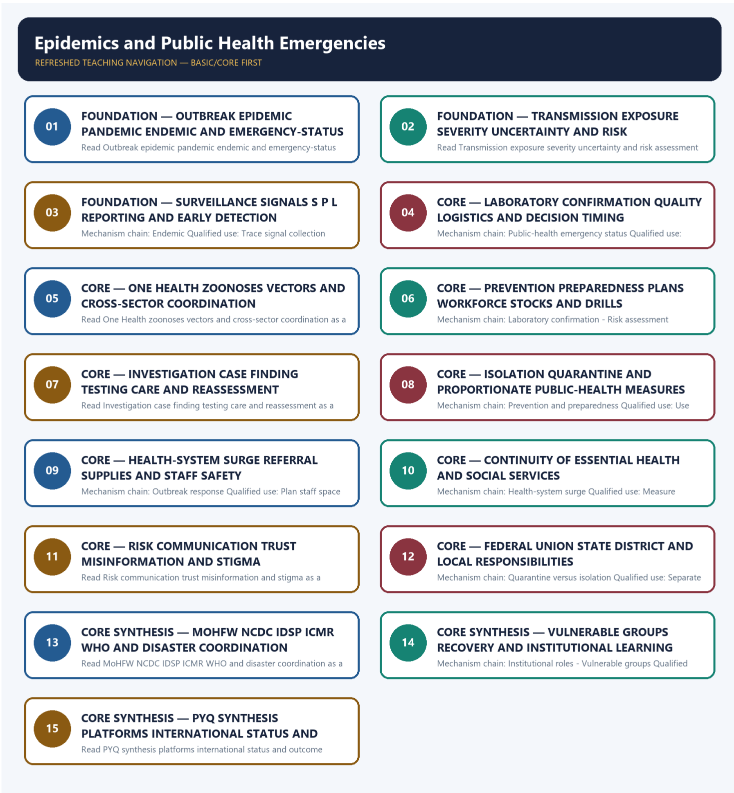
SOLVED PRACTICE WORKBOOK

Epidemics and Public Health Emergencies - Solved Practice Workbook

UPSC complete learning session | Foundation to advanced | Concepts, evidence, practice and answer writing



Original deterministic study visual; labels are explanatory, not textual quotations.

Source order: repository knowledge -> official current linkage -> clearly marked analysis. No fabricated PYQs or statistics.

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Major learning parts and meaningful subtopics are listed in document order. Page numbers are generated from the final PDF layout; PDF bookmarks mirror the same hierarchy.

Epidemics and Public Health Emergencies - Solved Practice Workbook

BASIC MCQS / REMEDIATION

Q1. Which statement correctly identifies Outbreak?

A. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. B. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. C. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. D. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone.

Answer: A. Explanation: An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q2. Which option preserves the risk or institutional boundary of Outbreak?

A. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. B. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. C. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. D. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks.

Answer: B. Explanation: An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q3. Which statement uses Outbreak without changing its hazard, mandate or status?

A. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. B. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. C. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. D. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks.

Answer: C. Explanation: An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q4. Which option avoids the standard UPSC close-option trap about Outbreak?

A. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. B. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. C. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. D. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label.

Answer: D. Explanation: An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q5. Which statement correctly identifies Epidemic?

A. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. B. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. C. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. D. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks.

Answer: A. Explanation: An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q6. Which option preserves the risk or institutional boundary of Epidemic?

A. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. B. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. C. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. D. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks.

Answer: B. Explanation: An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q7. Which statement uses Epidemic without changing its hazard, mandate or status?

A. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. B. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. C. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. D. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals.

Answer: C. Explanation: An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q8. Which option avoids the standard UPSC close-option trap about Epidemic?

A. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. B. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. C. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. D. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread.

Answer: D. Explanation: An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q9. Which statement correctly identifies Pandemic?

A. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. B. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. C. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. D. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests.

Answer: A. Explanation: A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q10. Which option preserves the risk or institutional boundary of Pandemic?

A. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. B. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. C. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. D. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals.

Answer: B. Explanation: A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q11. Which statement uses Pandemic without changing its hazard, mandate or status?

A. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. B. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. C. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. D. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures.

Answer: C. Explanation: A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q12. Which option avoids the standard UPSC close-option trap about Pandemic?

A. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. B. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. C. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. D. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone.

Answer: D. Explanation: A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q13. Which statement correctly identifies Endemic?

A. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. B. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. C. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. D. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals.

Answer: A. Explanation: An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q14. Which option preserves the risk or institutional boundary of Endemic?

A. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. B. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. C. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. D. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action.

Answer: B. Explanation: An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q15. Which statement uses Endemic without changing its hazard, mandate or status?

A. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. B. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. C. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. D. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures.

Answer: C. Explanation: An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q16. Which option avoids the standard UPSC close-option trap about Endemic?

A. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. B. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. C. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. D. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks.

Answer: D. Explanation: An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q17. Which statement correctly identifies Public-health emergency status?

A. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. B. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. C. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. D. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals.

Answer: A. Explanation: A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q18. Which option preserves the risk or institutional boundary of Public-health emergency status?

A. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. B. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. C. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. D. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability.

Answer: B. Explanation: A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q19. Which statement uses Public-health emergency status without changing its hazard, mandate or status?

A. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. B. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. C. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. D. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures.

Answer: C. Explanation: A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q20. Which option avoids the standard UPSC close-option trap about Public-health emergency status?

A. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. B. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. C. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. D. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests.

Answer: D. Explanation: A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q21. Which statement correctly identifies Surveillance?

A. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. B. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. C. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. D. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability.

Answer: A. Explanation: Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q22. Which option preserves the risk or institutional boundary of Surveillance?

A. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. B. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. C. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. D. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures.

Answer: B. Explanation: Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q23. Which statement uses Surveillance without changing its hazard, mandate or status?

A. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. B. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. C. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. D. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication.

Answer: C. Explanation: Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q24. Which option avoids the standard UPSC close-option trap about Surveillance?

A. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. B. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. C. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. D. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals.

Answer: D. Explanation: Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q25. Which statement correctly identifies Laboratory confirmation?

A. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. B. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. C. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. D. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability.

Answer: A. Explanation: Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q26. Which option preserves the risk or institutional boundary of Laboratory confirmation?

A. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. B. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. C. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. D. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment.

Answer: B. Explanation: Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q27. Which statement uses Laboratory confirmation without changing its hazard, mandate or status?

A. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. B. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. C. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. D. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care.

Answer: C. Explanation: Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q28. Which option avoids the standard UPSC close-option trap about Laboratory confirmation?

A. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. B. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. C. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. D. Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action.

Answer: D. Explanation: Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q29. Which statement correctly identifies Risk assessment?

A. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. B. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. C. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. D. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability.

Answer: A. Explanation: Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q30. Which option preserves the risk or institutional boundary of Risk assessment?

A. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. B. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. C. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. D. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care.

Answer: B. Explanation: Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q31. Which statement uses Risk assessment without changing its hazard, mandate or status?

A. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. B. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. C. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. D. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care.

Answer: C. Explanation: Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q32. Which option avoids the standard UPSC close-option trap about Risk assessment?

A. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. B. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. C. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. D. Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures.

Answer: D. Explanation: Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q33. Which statement correctly identifies One Health?

A. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. B. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. C. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. D. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment.

Answer: A. Explanation: One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q34. Which option preserves the risk or institutional boundary of One Health?

A. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. B. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. C. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. D. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care.

Answer: B. Explanation: One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q35. Which statement uses One Health without changing its hazard, mandate or status?

A. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. B. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. C. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. D. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty.

Answer: C. Explanation: One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q36. Which option avoids the standard UPSC close-option trap about One Health?

A. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. B. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. C. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. D. One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability.

Answer: D. Explanation: One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q37. Which statement correctly identifies Prevention and preparedness?

A. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. B. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. C. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. D. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm.

Answer: A. Explanation: Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q38. Which option preserves the risk or institutional boundary of Prevention and preparedness?

A. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. B. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. C. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. D. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty.

Answer: B. Explanation: Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q39. Which statement uses Prevention and preparedness without changing its hazard, mandate or status?

A. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. B. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. C. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. D. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm.

Answer: C. Explanation: Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q40. Which option avoids the standard UPSC close-option trap about Prevention and preparedness?

A. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. B. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. C. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. D. Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication.

Answer: D. Explanation: Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q41. Which statement correctly identifies Outbreak response?

A. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. B. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. C. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. D. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care.

Answer: A. Explanation: Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q42. Which option preserves the risk or institutional boundary of Outbreak response?

A. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. B. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. C. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. D. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols.

Answer: B. Explanation: Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q43. Which statement uses Outbreak response without changing its hazard, mandate or status?

A. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. B. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. C. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. D. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level.

Answer: C. Explanation: Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q44. Which option avoids the standard UPSC close-option trap about Outbreak response?

A. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. B. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. C. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. D. Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment.

Answer: D. Explanation: Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q45. Which statement correctly identifies Health-system surge?

A. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. B. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. C. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. D. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols.

Answer: A. Explanation: Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q46. Which option preserves the risk or institutional boundary of Health-system surge?

A. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. B. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. C. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. D. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level.

Answer: B. Explanation: Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q47. Which statement uses Health-system surge without changing its hazard, mandate or status?

A. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. B. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. C. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. D. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols.

Answer: C. Explanation: Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q48. Which option avoids the standard UPSC close-option trap about Health-system surge?

A. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. B. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. C. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. D. Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care.

Answer: D. Explanation: Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q49. Which statement correctly identifies Continuity of essential services?

A. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. B. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. C. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. D. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols.

Answer: A. Explanation: A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q50. Which option preserves the risk or institutional boundary of Continuity of essential services?

A. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. B. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. C. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. D. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols.

Answer: B. Explanation: A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q51. Which statement uses Continuity of essential services without changing its hazard, mandate or status?

A. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. B. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. C. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. D. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers.

Answer: C. Explanation: A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q52. Which option avoids the standard UPSC close-option trap about Continuity of essential services?

A. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. B. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. C. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. D. A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm.

Answer: D. Explanation: A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q53. Which statement correctly identifies Risk communication?

A. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. B. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. C. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. D. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols.

Answer: A. Explanation: Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q54. Which option preserves the risk or institutional boundary of Risk communication?

A. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. B. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. C. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. D. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers.

Answer: B. Explanation: Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q55. Which statement uses Risk communication without changing its hazard, mandate or status?

A. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. B. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. C. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. D. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers.

Answer: C. Explanation: Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q56. Which option avoids the standard UPSC close-option trap about Risk communication?

A. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. B. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. C. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. D. Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty.

Answer: D. Explanation: Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q57. Which statement correctly identifies Quarantine versus isolation?

A. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. B. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. C. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. D. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework.

Answer: A. Explanation: Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q58. Which option preserves the risk or institutional boundary of Quarantine versus isolation?

A. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. B. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. C. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. D. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons.

Answer: B. Explanation: Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q59. Which statement uses Quarantine versus isolation without changing its hazard, mandate or status?

A. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. B. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. C. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. D. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence.

Answer: C. Explanation: Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q60. Which option avoids the standard UPSC close-option trap about Quarantine versus isolation?

A. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. B. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. C. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. D. Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols.

Answer: D. Explanation: Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q61. Which statement correctly identifies Federal and local roles?

A. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. B. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. C. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. D. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons.

Answer: A. Explanation: Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q62. Which option preserves the risk or institutional boundary of Federal and local roles?

A. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. B. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. C. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. D. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers.

Answer: B. Explanation: Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q63. Which statement uses Federal and local roles without changing its hazard, mandate or status?

A. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. B. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. C. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. D. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons.

Answer: C. Explanation: Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q64. Which option avoids the standard UPSC close-option trap about Federal and local roles?

A. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. B. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. C. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. D. Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level.

Answer: D. Explanation: Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q65. Which statement correctly identifies Institutional roles?

A. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. B. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. C. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. D. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons.

Answer: A. Explanation: MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q66. Which option preserves the risk or institutional boundary of Institutional roles?

A. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. B. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. C. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. D. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence.

Answer: B. Explanation: MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q67. Which statement uses Institutional roles without changing its hazard, mandate or status?

A. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. B. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. C. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. D. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence.

Answer: C. Explanation: MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and

WHO coordinates the international IHR framework. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q68. Which option avoids the standard UPSC close-option trap about Institutional roles?

A. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. B. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. C. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. D. MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework.

Answer: D. Explanation: MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q69. Which statement correctly identifies Vulnerable groups?

A. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. B. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. C. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. D. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence.

Answer: A. Explanation: Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q70. Which option preserves the risk or institutional boundary of Vulnerable groups?

A. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. B. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. C. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. D. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label.

Answer: B. Explanation: Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q71. Which statement uses Vulnerable groups without changing its hazard, mandate or status?

A. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. B. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. C. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. D. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread.

Answer: C. Explanation: Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital,

language, income or care barriers. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q72. Which option avoids the standard UPSC close-option trap about Vulnerable groups?

A. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. B. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. C. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. D. Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers.

Answer: D. Explanation: Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q73. Which statement correctly identifies Misinformation and recovery?

A. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. B. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. C. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. D. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread.

Answer: A. Explanation: Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q74. Which option preserves the risk or institutional boundary of Misinformation and recovery?

A. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. B. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. C. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. D. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label.

Answer: B. Explanation: Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q75. Which statement uses Misinformation and recovery without changing its hazard, mandate or status?

A. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. B. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. C. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. D. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks.

Answer: C. Explanation: Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review

excess harm and institutionalise lessons. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q76. Which option avoids the standard UPSC close-option trap about Misinformation and recovery?

A. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. B. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. C. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. D. Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons.

Answer: D. Explanation: Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q77. Which statement correctly identifies Platform-outcome firewall?

A. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. B. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. C. An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. D. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread.

Answer: A. Explanation: A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q78. Which option preserves the risk or institutional boundary of Platform-outcome firewall?

A. An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. B. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. C. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. D. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks.

Answer: B. Explanation: A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q79. Which statement uses Platform-outcome firewall without changing its hazard, mandate or status?

A. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. B. A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. C. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. D. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks.

Answer: C. Explanation: A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

Q80. Which option avoids the standard UPSC close-option trap about Platform-outcome firewall?

A. Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. B. A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. C. An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. D. A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence.

Answer: D. Explanation: A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The other options change the hazard or risk category, institutional owner, legal instrument, warning stage, evidence rung or implementation status.

PYQS AND ANSWER PRACTICE

AUDITED TOPIC-SPECIFIC PYQ OWNERSHIP

The 2020 GS-III technology-in-COVID card is the verified direct route. The 2020 GS-II WHO card is cross-paper and remains global-governance-owned; the 2024 resilience card is an explicit support route.

PYQ DEMAND CARD 1 - 2020 GS-III

Demand: Give an account of technology used in COVID-19 pandemic management and its advancements.

Status: Verified direct routing: Give an account of · 15 marks · 250 words; organise technology by surveillance, diagnosis, communication, care and delivery functions and preserve equity and outcome limits.

Model solution: Surveillance: Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Laboratory confirmation:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Risk assessment:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Outbreak response:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Health-system surge:** Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. **Risk communication:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Institutional roles:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Vulnerable groups:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Platform-outcome firewall:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The solution therefore answers the routed demand without inventing an official model answer or an objective answer key.

Demand decoding: The directive **answer** requires a direct position on ‘PYQ DEMAND CARD 1 - 2020 GS-III’, each clause, risk mechanism, named Indian law/institution/event, dated status, implementation and qualification.

Detailed examiner-grade model answer:

Introduction and thesis: Surveillance: Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Laboratory confirmation:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Risk assessment:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Outbreak response:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Health-system surge:** Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. **Risk communication:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Institutional roles:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Vulnerable groups:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Platform-outcome firewall:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The solution therefore answers the routed demand without inventing an official model answer or an objective answer key.

Analytical body:

1. **Claim:** Demand: Give an account of technology used in COVID-19 pandemic management and its advancements. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
2. **Claim:** Status: Verified direct routing: Give an account of · 15 marks · 250 words; organise technology by surveillance, diagnosis, communication, care and delivery functions and preserve equity and outcome limits. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.

Counter-position / limit: An Act, plan, guideline, scheme, forecast, warning, drill, team, sanctioned capacity, allocation or dispatch cannot alone establish implementation, evacuation, expenditure, reduced loss, equitable recovery or resilience; test mandate, process, status and evidence.

Qualified conclusion: Surveillance: Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Laboratory confirmation:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Risk assessment:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Outbreak response:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Health-system surge:** Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. **Risk communication:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Institutional roles:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Vulnerable groups:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Platform-outcome firewall:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The solution therefore answers the routed demand without inventing an official model answer or an objective answer key.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing hazard -> exposure/vulnerability/capacity -> risk/impact -> prevention/mitigation/preparedness -> response/recovery/BBB; write four to seven claim -> named evidence/example -> analysis -> qualification points; reserve the final minute for source, date, unit, status, mandate and event/inference checks.

Why this earns marks: The answer obeys the directive, uses India-centric evidence, and preserves risk terms, legal character, institutional mandate, warning/cycle stage, finance status, implementation and resilience distinctions.

How to improve this answer: For ‘PYQ DEMAND CARD 1 - 2020 GS-III’, replace the weakest generic point with one exact Indian mechanism, institution/guideline/event, dated source and unit, implementation bottleneck, local-capacity or inclusion safeguard and answer-specific qualification.

PYQ DEMAND CARD 2 - 2020 GS-II

Demand: Discuss WHO's role in global health security during the COVID-19 pandemic.

Status: Verified cross-paper route owned by International Relations/global governance; this card contributes only IHR status, national institutions and sovereignty boundaries.

Model solution: Public-health emergency status: A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Risk communication:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Institutional roles:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Platform-outcome firewall:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The solution therefore answers the routed demand without inventing an official model answer or an objective answer key.

Demand decoding: The directive **answer** requires a direct position on ‘PYQ DEMAND CARD 2 - 2020 GS-II’, each clause, risk mechanism, named Indian law/institution/event, dated status, implementation and qualification.

Detailed examiner-grade model answer:

Introduction and thesis: Public-health emergency status: A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Risk communication:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Institutional roles:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Platform-outcome firewall:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The solution therefore answers the routed demand without inventing an official model answer or an objective answer key.

Analytical body:

1. **Claim:** Demand: Discuss WHO's role in global health security during the COVID-19 pandemic. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
2. **Claim:** Status: Verified cross-paper route owned by International Relations/global governance; this card contributes only IHR status, national institutions and sovereignty boundaries. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source.

Analysis: Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.

Counter-position / limit: An Act, plan, guideline, scheme, forecast, warning, drill, team, sanctioned capacity, allocation or dispatch cannot alone establish implementation, evacuation, expenditure, reduced loss, equitable recovery or resilience; test mandate, process, status and evidence.

Qualified conclusion: Public-health emergency status: A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Risk communication:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Institutional roles:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Platform-outcome firewall:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The solution therefore answers the routed demand without inventing an official model answer or an objective answer key.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing hazard -> exposure/vulnerability/capacity -> risk/impact -> prevention/mitigation/preparedness -> response/recovery/BBB; write four to seven claim -> named evidence/example -> analysis -> qualification points; reserve the final minute for source, date, unit, status, mandate and event/inference checks.

Why this earns marks: The answer obeys the directive, uses India-centric evidence, and preserves risk terms, legal character, institutional mandate, warning/cycle stage, finance status, implementation and resilience distinctions.

How to improve this answer: For 'PYQ DEMAND CARD 2 - 2020 GS-II', replace the weakest generic point with one exact Indian mechanism, institution/guideline/event, dated source and unit, implementation bottleneck, local-capacity or inclusion safeguard and answer-specific qualification.

PYQ DEMAND CARD 3 - 2024 GS-III

Demand: Describe disaster resilience, its determination and the Sendai Framework elements.

Status: Verified support route, not a public-health-emergency-specific PYQ; surveillance, surge, continuity, communication and recovery are bounded resilience illustrations.

Model solution: Surveillance: Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **One Health:** One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. **Prevention and preparedness:** Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. **Health-system surge:** Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. **Continuity of essential services:** A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. **Risk communication:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Misinformation and recovery:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Platform-outcome firewall:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The solution therefore answers the routed demand without inventing an official model answer or an objective answer key.

Demand decoding: The directive **answer** requires a direct position on 'PYQ DEMAND CARD 3 - 2024 GS-III', each clause, risk mechanism, named Indian law/institution/event, dated status, implementation and qualification.

Detailed examiner-grade model answer:

Introduction and thesis: Surveillance: Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **One Health:** One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. **Prevention and preparedness:** Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. **Health-system surge:** Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. **Continuity of essential services:** A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. **Risk communication:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Misinformation and recovery:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Platform-outcome firewall:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The solution therefore answers the routed demand without inventing an official model answer or an objective answer key.

Analytical body:

1. **Claim:** Demand: Describe disaster resilience, its determination and the Sendai Framework elements. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
2. **Claim:** Status: Verified support route, not a public-health-emergency-specific PYQ; surveillance, surge, continuity, communication and recovery are bounded resilience illustrations. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.

Counter-position / limit: An Act, plan, guideline, scheme, forecast, warning, drill, team, sanctioned capacity, allocation or dispatch cannot alone establish implementation, evacuation, expenditure, reduced loss, equitable recovery or resilience; test mandate, process, status and evidence.

Qualified conclusion: Surveillance: Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **One Health:** One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. **Prevention and preparedness:** Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. **Health-system surge:** Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. **Continuity of essential services:** A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. **Risk communication:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Misinformation and recovery:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Platform-outcome firewall:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. The solution therefore answers the routed demand without inventing an official model answer or an objective answer key.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing hazard -> exposure/vulnerability/capacity -> risk/impact -> prevention/mitigation/preparedness -> response/recovery/BBB; write four to seven claim -> named evidence/example -> analysis -> qualification points; reserve the final minute for source, date, unit, status, mandate and event/inference checks.

Why this earns marks: The answer obeys the directive, uses India-centric evidence, and preserves risk terms, legal character, institutional mandate, warning/cycle stage, finance status, implementation and resilience distinctions.

How to improve this answer: For ‘PYQ DEMAND CARD 3 - 2024 GS-III’, replace the weakest generic point with one exact Indian mechanism, institution/guideline/event, dated source and unit, implementation bottleneck, local-capacity or inclusion safeguard and answer-specific qualification.

ORIGINAL MAINS 1 - 10 MARKS

Question: Distinguish outbreak, epidemic, pandemic, endemic and public-health emergency statuses. Answer in about 150 words.

Model thesis: Claim: Outbreak. **Named evidence/example:** An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Epidemic. **Named evidence/example:** An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Pandemic. **Named evidence/example:** A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Endemic. **Named evidence/example:** An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Public-health emergency status. **Named evidence/example:** A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Claim -> named evidence -> analysis -> qualification:

- An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label.
- An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread.
- A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone.
- An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks.
- A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests.

Qualified conclusion: Claim: Outbreak. **Named evidence/example:** An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Epidemic. **Named evidence/example:** An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Pandemic. **Named evidence/example:** A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Endemic. **Named evidence/example:** An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Public-health emergency status. **Named evidence/example:** A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Demand decoding: The directive **answer** requires a direct position on ‘Distinguish outbreak, epidemic, pandemic, endemic and public-health emergency statuses.…’, each clause, risk mechanism, named Indian law/institution/event, dated status, implementation and qualification.

Detailed examiner-grade model answer:

Introduction and thesis: Claim: Outbreak. **Named evidence/example:** An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Epidemic. **Named evidence/example:** An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Pandemic. **Named evidence/example:** A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Endemic. **Named evidence/example:** An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Public-health emergency status. **Named evidence/example:** A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Analytical body:

1. **Claim:** An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit,

event/inference boundary or residual risk.

2. **Claim:** An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
3. **Claim:** A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
4. **Claim:** An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
5. **Claim:** A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.

Counter-position / limit: An Act, plan, guideline, scheme, forecast, warning, drill, team, sanctioned capacity, allocation or dispatch cannot alone establish implementation, evacuation, expenditure, reduced loss, equitable recovery or resilience; test mandate, process, status and evidence.

Qualified conclusion: Claim: Outbreak. **Named evidence/example:** An outbreak is the occurrence of disease cases above what is normally expected in a limited place, population or period; scale and baseline determine the label. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Epidemic. **Named evidence/example:** An epidemic is occurrence of disease above the expected level in a community, region or population; it does not require international spread. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Pandemic. **Named evidence/example:** A pandemic is an epidemic with sustained spread across countries or continents and substantial population reach; severity is not defined by geography alone. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Endemic. **Named evidence/example:** An endemic disease has a persistent or expected presence in a particular area or population; endemicity does not mean harmlessness or absence of outbreaks. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Public-health emergency status. **Named evidence/example:** A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Executable exam-length answer / compression plan: For 10 marks, spend one-sixth of the time decoding the directive and drawing hazard -> exposure/vulnerability/capacity -> risk/impact -> prevention/mitigation/preparedness -> response/recovery/BBB; write four to seven claim -> named evidence/example -> analysis -> qualification points; reserve the final minute for source, date, unit, status, mandate and event/inference checks.

Why this earns marks: The answer obeys the directive, uses India-centric evidence, and preserves risk terms, legal character, institutional mandate, warning/cycle stage, finance status, implementation and resilience distinctions.

How to improve this answer: For 'Distinguish outbreak, epidemic, pandemic, endemic and public-health emergency statuses.…', replace the weakest generic point with one exact Indian mechanism, institution/guideline/event, dated source and unit, implementation bottleneck, local-capacity or inclusion safeguard and answer-specific qualification.

ORIGINAL MAINS 2 - 10 MARKS

Question: Differentiate quarantine and isolation and state their governance safeguards. Answer in about 150 words.

Model thesis: Claim: Outbreak response. **Named evidence/example:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Quarantine versus isolation. **Named evidence/example:** Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Vulnerable groups. **Named evidence/example:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Claim -> named evidence -> analysis -> qualification:

- Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment.
- Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols.
- Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers.

Qualified conclusion: Claim: Outbreak response. **Named evidence/example:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Quarantine versus isolation. **Named evidence/example:** Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Vulnerable groups. **Named evidence/example:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Demand decoding: The directive **answer** requires a direct position on 'Differentiate quarantine and isolation and state their governance safeguards. Answer in about…', each clause, risk mechanism, named Indian law/institution/event, dated status, implementation and qualification.

Detailed examiner-grade model answer:

Introduction and thesis: Claim: Outbreak response. **Named evidence/example:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Quarantine versus isolation. **Named evidence/example:** Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Vulnerable groups. **Named evidence/example:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Analytical body:

1. **Claim:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
2. **Claim:** Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
3. **Claim:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.

Counter-position / limit: An Act, plan, guideline, scheme, forecast, warning, drill, team, sanctioned capacity, allocation or dispatch cannot alone establish implementation, evacuation, expenditure, reduced loss, equitable recovery or resilience; test mandate, process, status and evidence.

Qualified conclusion: Claim: Outbreak response. **Named evidence/example:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Quarantine versus isolation. **Named evidence/example:** Quarantine separates or restricts movement of persons who may have been exposed but are not known to be infected; isolation separates infected or ill persons under applicable public-health protocols. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Vulnerable groups. **Named evidence/example:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Executable exam-length answer / compression plan: For 10 marks, spend one-sixth of the time decoding the directive and drawing hazard -> exposure/vulnerability/capacity -> risk/impact -> prevention/mitigation/preparedness -> response/recovery/BBB; write four to seven claim -> named evidence/example -> analysis -> qualification points; reserve the final minute for source, date, unit, status, mandate and event/inference checks.

Why this earns marks: The answer obeys the directive, uses India-centric evidence, and preserves risk terms, legal character, institutional mandate, warning/cycle stage, finance status, implementation and resilience distinctions.

How to improve this answer: For ‘Differentiate quarantine and isolation and state their governance safeguards. Answer in about…’, replace the weakest generic point with one exact Indian mechanism, institution/guideline/event, dated source and unit, implementation bottleneck, local-capacity or inclusion safeguard and answer-specific qualification.

ORIGINAL MAINS 3 - 15 MARKS

Question: Explain the surveillance, laboratory-confirmation and risk-assessment chain for outbreak response. Answer in about 250 words.

Model thesis: Claim: Surveillance. **Named evidence/example:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Laboratory confirmation. **Named evidence/example:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk assessment. **Named evidence/example:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Outbreak response. **Named evidence/example:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Claim -> named evidence -> analysis -> qualification:

- Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals.
- Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action.
- Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures.
- Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment.

- A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence.

Qualified conclusion: Claim: Surveillance. **Named evidence/example:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Laboratory confirmation. **Named evidence/example:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk assessment. **Named evidence/example:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Outbreak response. **Named evidence/example:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Demand decoding: The directive **explain** requires a direct position on ‘Explain the surveillance, laboratory-confirmation and risk-assessment chain for outbreak…’, each clause, risk mechanism, named Indian law/institution/event, dated status, implementation and qualification.

Detailed examiner-grade model answer:

Introduction and thesis: Claim: Surveillance. **Named evidence/example:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Laboratory confirmation. **Named evidence/example:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk assessment. **Named evidence/example:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Outbreak response. **Named evidence/example:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Analytical body:

1. **Claim:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
2. **Claim:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
3. **Claim:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
4. **Claim:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
5. **Claim:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.

Counter-position / limit: An Act, plan, guideline, scheme, forecast, warning, drill, team, sanctioned capacity, allocation or dispatch cannot alone establish implementation, evacuation, expenditure, reduced loss, equitable recovery or resilience; test mandate, process, status and evidence.

Qualified conclusion: Claim: Surveillance. **Named evidence/example:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Laboratory confirmation. **Named evidence/example:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk assessment. **Named evidence/example:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Outbreak response. **Named evidence/example:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing hazard -> exposure/vulnerability/capacity -> risk/impact -> prevention/mitigation/preparedness -> response/recovery/BBB; write four to seven claim -> named evidence/example -> analysis -> qualification points; reserve the final minute for source, date, unit, status, mandate and event/inference checks.

Why this earns marks: The answer obeys the directive, uses India-centric evidence, and preserves risk terms, legal character, institutional mandate, warning/cycle stage, finance status, implementation and resilience distinctions.

How to improve this answer: For ‘Explain the surveillance, laboratory-confirmation and risk-assessment chain for outbreak…’, replace the weakest generic point with one exact Indian mechanism, institution/guideline/event, dated source and unit, implementation bottleneck, local-capacity or inclusion safeguard and answer-specific qualification.

ORIGINAL MAINS 4 - 15 MARKS

Question: Analyse One Health and federal coordination for zoonotic and epidemic threats. Answer in about 250 words.

Model thesis: Claim: One Health. **Named evidence/example:** One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Federal and local roles. **Named evidence/example:** Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Institutional roles. **Named evidence/example:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Claim -> named evidence -> analysis -> qualification:

- One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability.
- Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and

multi-level.

- MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework.
- A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence.

Qualified conclusion: Claim: One Health. **Named evidence/example:** One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Federal and local roles. **Named evidence/example:** Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Institutional roles. **Named evidence/example:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Demand decoding: The directive **analyse** requires a direct position on ‘Analyse One Health and federal coordination for zoonotic and epidemic threats. Answer in…’, each clause, risk mechanism, named Indian law/institution/event, dated status, implementation and qualification.

Detailed examiner-grade model answer:

Introduction and thesis: Claim: One Health. **Named evidence/example:** One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Federal and local roles. **Named evidence/example:** Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Institutional roles. **Named evidence/example:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Analytical body:

1. **Claim:** One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
2. **Claim:** Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
3. **Claim:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
4. **Claim:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.

Counter-position / limit: An Act, plan, guideline, scheme, forecast, warning, drill, team, sanctioned capacity, allocation or dispatch cannot alone establish implementation, evacuation, expenditure, reduced loss, equitable recovery or resilience; test mandate, process, status and evidence.

Qualified conclusion: Claim: One Health. **Named evidence/example:** One Health coordinates human, animal and environmental health knowledge and action, especially for zoonotic and vector-borne threats, while preserving sector-specific expertise and accountability. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Federal and local roles. **Named evidence/example:** Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Institutional roles. **Named evidence/example:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Executable exam-length answer / compression plan: For 15 marks, spend one-sixth of the time decoding the directive and drawing hazard -> exposure/vulnerability/capacity -> risk/impact -> prevention/mitigation/preparedness

-> response/recovery/BBB; write four to seven claim -> named evidence/example -> analysis -> qualification points; reserve the final minute for source, date, unit, status, mandate and event/inference checks.

Why this earns marks: The answer obeys the directive, uses India-centric evidence, and preserves risk terms, legal character, institutional mandate, warning/cycle stage, finance status, implementation and resilience distinctions.

How to improve this answer: For ‘Analyse One Health and federal coordination for zoonotic and epidemic threats. Answer in…’, replace the weakest generic point with one exact Indian mechanism, institution/guideline/event, dated source and unit, implementation bottleneck, local-capacity or inclusion safeguard and answer-specific qualification.

ORIGINAL MAINS 5 - 20 MARKS

Question: Design a public-health-emergency framework covering prevention, surge, continuity, communication and vulnerable groups. Answer in about 300 words.

Model thesis: Claim: Surveillance. **Named evidence/example:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Laboratory confirmation. **Named evidence/example:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk assessment. **Named evidence/example:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Prevention and preparedness. **Named evidence/example:** Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Outbreak response. **Named evidence/example:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Health-system surge. **Named evidence/example:** Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Continuity of essential services. **Named evidence/example:** A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk communication. **Named evidence/example:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Vulnerable groups. **Named evidence/example:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Misinformation and recovery. **Named evidence/example:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Claim -> named evidence -> analysis -> qualification:

- Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals.
- Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action.
- Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures.
- Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication.
- Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment.
- Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care.
- A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm.
- Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty.
- Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers.
- Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons.
- A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence.

Qualified conclusion: Claim: Surveillance. **Named evidence/example:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Laboratory confirmation. **Named evidence/example:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk assessment. **Named evidence/example:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Prevention and preparedness. **Named evidence/example:** Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Outbreak response. **Named evidence/example:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Health-system surge. **Named evidence/example:** Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Continuity of essential services. **Named evidence/example:** A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk communication. **Named evidence/example:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Vulnerable groups. **Named evidence/example:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Misinformation and recovery. **Named evidence/example:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Demand decoding: The directive **answer** requires a direct position on ‘Design a public-health-emergency framework covering prevention, surge, continuity,…’, each clause, risk mechanism, named Indian law/institution/event, dated status, implementation and qualification.

Detailed examiner-grade model answer:

Introduction and thesis: Claim: Surveillance. **Named evidence/example:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Laboratory confirmation.

Named evidence/example: Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk assessment. **Named evidence/example:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Prevention and preparedness. **Named evidence/example:** Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Outbreak response. **Named evidence/example:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Health-system surge. **Named evidence/example:** Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Continuity of essential services. **Named evidence/example:** A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk communication. **Named evidence/example:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Vulnerable groups. **Named evidence/example:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Misinformation and recovery. **Named evidence/example:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Analytical body:

1. **Claim:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Named evidence/example:** Identify the exact

Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.

2. **Claim:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
3. **Claim:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
4. **Claim:** Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
5. **Claim:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
6. **Claim:** Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
7. **Claim:** A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
8. **Claim:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
9. **Claim:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status,

mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.

10. **Claim:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
11. **Claim:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.

Counter-position / limit: An Act, plan, guideline, scheme, forecast, warning, drill, team, sanctioned capacity, allocation or dispatch cannot alone establish implementation, evacuation, expenditure, reduced loss, equitable recovery or resilience; test mandate, process, status and evidence.

Qualified conclusion: Claim: Surveillance. **Named evidence/example:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Laboratory confirmation. **Named evidence/example:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk assessment. **Named evidence/example:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Prevention and preparedness. **Named evidence/example:** Prevention and preparedness include water, sanitation, vector control, infection prevention, routine immunisation where applicable, stock and workforce planning, laboratories, plans, training, drills and trusted communication. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Outbreak response. **Named evidence/example:** Response links verification, investigation, case finding, testing, clinical care, isolation where indicated, contact or exposure management, community measures, logistics and iterative reassessment. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Health-system surge. **Named evidence/example:** Surge capacity covers trained staff, beds and referral, laboratories, medicines and supplies, oxygen or other clinical support where relevant, transport, information systems and staff safety without abandoning routine care. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Continuity of essential services. **Named evidence/example:** A health emergency plan must preserve maternal, child, chronic-disease, emergency and other essential services as far as possible because response measures can create indirect harm. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk communication. **Named evidence/example:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Vulnerable groups. **Named evidence/example:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Misinformation and recovery. **Named evidence/example:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Executable exam-length answer / compression plan: For 20 marks, spend one-sixth of the time decoding the directive and drawing hazard -> exposure/vulnerability/capacity -> risk/impact -> prevention/mitigation/preparedness -> response/recovery/BBB; write four to seven claim -> named evidence/example -> analysis -> qualification points; reserve the final minute for source, date, unit, status, mandate and event/inference checks.

Why this earns marks: The answer obeys the directive, uses India-centric evidence, and preserves risk terms, legal character, institutional mandate, warning/cycle stage, finance status, implementation and resilience distinctions.

How to improve this answer: For ‘Design a public-health-emergency framework covering prevention, surge, continuity,…’, replace the weakest generic point with one exact Indian mechanism, institution/guideline/event, dated source and unit, implementation bottleneck, local-capacity or inclusion safeguard and answer-specific qualification.

ORIGINAL MAINS 6 - 20 MARKS

Question: Critically evaluate whether laws, platforms and international agreements ensure pandemic resilience. Answer in about 300 words.

Model thesis: Claim: Public-health emergency status. **Named evidence/example:** A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Surveillance. **Named evidence/example:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Laboratory confirmation. **Named evidence/example:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk assessment. **Named evidence/example:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk communication. **Named evidence/example:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Federal and local roles. **Named evidence/example:** Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Institutional roles. **Named evidence/example:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Vulnerable groups. **Named evidence/example:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Misinformation and recovery. **Named evidence/example:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Claim -> named evidence -> analysis -> qualification:

- A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests.
- Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals.
- Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action.
- Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures.
- Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty.
- Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level.
- MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework.
- Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers.

- Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons.
- A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence.

Qualified conclusion: Claim: Public-health emergency status. **Named evidence/example:** A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Surveillance. **Named evidence/example:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Laboratory confirmation. **Named evidence/example:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk assessment. **Named evidence/example:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk communication. **Named evidence/example:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Federal and local roles. **Named evidence/example:** Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Institutional roles. **Named evidence/example:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Vulnerable groups. **Named evidence/example:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Misinformation and recovery. **Named evidence/example:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Demand decoding: The directive **evaluate** requires a direct position on ‘Critically evaluate whether laws, platforms and international agreements ensure pandemic…’, each clause, risk mechanism, named Indian law/institution/event, dated status, implementation and qualification.

Detailed examiner-grade model answer:

Introduction and thesis: Claim: Public-health emergency status. **Named evidence/example:** A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Surveillance. **Named evidence/example:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Laboratory confirmation. **Named evidence/example:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk assessment. **Named evidence/example:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk communication. **Named evidence/example:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Federal and local roles. **Named evidence/example:** Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Institutional roles. **Named evidence/example:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Vulnerable groups. **Named evidence/example:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Misinformation and recovery. **Named evidence/example:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Analytical body:

1. **Claim:** A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
2. **Claim:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
3. **Claim:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
4. **Claim:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
5. **Claim:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
6. **Claim:** Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
7. **Claim:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
8. **Claim:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers.

Named evidence/example: Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.

9. **Claim:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.
10. **Claim:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Named evidence/example:** Identify the exact Indian law, institution, guideline, warning system, fund, plan or dated event owned by the source. **Analysis:** Connect hazard -> exposure/vulnerability/capacity -> disaster consequence -> competent prevention/response/recovery route. **Qualification:** State source/date/unit/status, mandate, uncertainty, implementation gap, inclusion limit, event/inference boundary or residual risk.

Counter-position / limit: An Act, plan, guideline, scheme, forecast, warning, drill, team, sanctioned capacity, allocation or dispatch cannot alone establish implementation, evacuation, expenditure, reduced loss, equitable recovery or resilience; test mandate, process, status and evidence.

Qualified conclusion: Claim: Public-health emergency status. **Named evidence/example:** A domestic public-health emergency, a Public Health Emergency of International Concern and the amended IHR's pandemic emergency are different legal or operational statuses with different authorities and tests. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Surveillance. **Named evidence/example:** Surveillance is continuous collection, analysis, interpretation and use of health information for action; it includes signals and trends rather than only confirmed case totals. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Laboratory confirmation. **Named evidence/example:** Laboratory evidence supports diagnosis and classification, but sampling, transport, quality, turnaround and interpretation determine whether confirmation can guide timely public-health action. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk assessment. **Named evidence/example:** Risk assessment integrates hazard, transmissibility or route, severity, exposure, vulnerable groups, uncertainty, health-system capacity and geographic or temporal context before selecting proportionate measures. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Risk communication. **Named evidence/example:** Risk communication should be timely, transparent, multilingual, accessible and updateable, explaining what is known, unknown, changing and expected from the public without blame or false certainty. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Federal and local roles. **Named evidence/example:** Health is principally a State field, while Union law, inter-State disease control, national standards, laboratories, border health and disaster coordination can support or direct specified functions; implementation remains local and multi-level. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Institutional roles. **Named evidence/example:** MoHFW provides national health leadership, NCDC and IDSP support surveillance and outbreak response, ICMR supports research and technical evidence, States deliver public health, and WHO coordinates the international IHR framework. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Vulnerable groups. **Named evidence/example:** Risk and access differ for older people, children, pregnant persons, persons with disabilities or chronic illness, health workers, migrants, crowded households and those facing digital, language, income or care barriers. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Misinformation and recovery. **Named evidence/example:** Rumours, stigma and information overload can undermine response; recovery should restore services, address interrupted care and livelihoods, support workers and communities, review excess harm and institutionalise lessons. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary. **Claim:** Platform-outcome firewall. **Named evidence/example:** A surveillance portal, laboratory, app, guideline, emergency declaration, agreement or vaccination platform proves a tool or status; early detection, equitable care, trust, continuity and reduced harm require separate evidence. **Analysis:** This fixes the hazard, risk or protection category, institutional mandate and verified evidence rung before drawing the policy implication. **Qualification:** The conclusion retains the owner's date, institution, legal character and the mandate-to-implementation-to-outcome boundary.

Executable exam-length answer / compression plan: For 20 marks, spend one-sixth of the time decoding the directive and drawing hazard -> exposure/vulnerability/capacity -> risk/impact -> prevention/mitigation/preparedness -> response/recovery/BBB; write four to seven claim -> named evidence/example -> analysis -> qualification points; reserve the final minute for source, date, unit, status, mandate and event/inference checks.

Why this earns marks: The answer obeys the directive, uses India-centric evidence, and preserves risk terms, legal character, institutional mandate, warning/cycle stage, finance status, implementation and resilience distinctions.

How to improve this answer: For ‘Critically evaluate whether laws, platforms and international agreements ensure pandemic…’, replace the weakest generic point with one exact Indian mechanism, institution/guideline/event, dated source and unit, implementation bottleneck, local-capacity or inclusion safeguard and answer-specific qualification.